LUITPARIA CANCER INSTITUTE
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Reg. No. AS/GHY/HOSP/2009/0076

OPERATION NOTE

Name: Bhupen Kalita Age / Sex: 63 / M IP No.: LCI/IP/26/2214

Date: 26/5/2026

Surgeon: Dr. Dipankar Saikia Assistants: Dr. Mridul Hazarika, Dr. Nabanita Deka

Anaesthesia: GA with double-lumen tube + thoracic epidural (Dr. Jyotishman Bora)

Pre-op diagnosis: Ca mid-thoracic oesophagus (SCC), post NACRT

Operation: Minimally invasive three-stage (McKeown) oesophagectomy with two-field lymphadenectomy, gastric conduit via the posterior mediastinum, left cervical oesophagogastric anastomosis and feeding jejunostomy

Findings

Thorax: residual fibrotic thickening of the mid-oesophagus, not fixed to the aorta or airway; subcarinal and paraoesophageal nodes enlarged. Abdomen: no liver or peritoneal deposits; no ascites.

Procedure in brief

- Thoracoscopic phase (prone): oesophagus mobilised from the thoracic inlet to the hiatus with en bloc paraoesophageal, subcarinal and right recurrent laryngeal nerve nodes; thoracic duct clipped.
- Laparoscopic phase: gastric mobilisation preserving the right gastroepiploic arcade; left gastric pedicle divided with clearance of station 7, 8 and 9 nodes; 4 cm wide gastric conduit fashioned.
- Cervical phase: left neck incision, conduit pulled up; stapled side-to-side oesophagogastric anastomosis.
- Feeding jejunostomy (Witzel) 20 cm from the DJ flexure. Right ICD 28 Fr and an abdominal drain placed.

Blood loss 250 mL; no transfusion. Duration 6 h 40 min.

Specimen (oesophagus with proximal stomach and separately labelled nodal stations) sent for HPE.

Post-op: elective overnight ventilation in SICU; NPO, FJ feeds from POD 1; ICD to underwater seal.

Dr. Dipankar Saikia
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Reg. No. ASMC-11962
Consultant Surgical Oncologist

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY